



The Corridor Outside Ward Three

Student copy (project or hand out)

1 I have memorized a corridor. Not a poem, not a prayer, a corridor: the third tile from the lift that lifts at one corner, the vending machine that only ever has the orange crisps left, the chair by the window where I learned to sleep sitting up. My daughter was born with esophageal atresia, which is a long way of saying the tube that should carry food from her mouth to her stomach was never finished, came into the world in two pieces that did not meet. She is one of roughly one in four thousand. I used to find that number comforting and then I stopped, because a number does not feed anybody.

2 What they do not tell you, in the early days, is how much of this is waiting. There is the surgery, yes, and there were several of them, and there is the gastrostomy tube, the little port in her belly through which she has taken nearly every meal of her life. I got fast at it. I can prime a feed in the dark, half asleep, the way you used to know the words to a song. What I never got used to was the watching: watching her watch other children eat, watching her learn that her hunger and her stomach were strangers to each other.

3 And there is a longer dread underneath the daily one. When the doctors first talked, carefully, about what a transplant might one day mean, they also said the word that has lived in me ever since: immunosuppression. Anti-rejection drugs, for life. Drugs that would leave her more open to every passing infection, and that carry, the literature is plain about this, a two to four fold higher risk of cancer down the years. You trade one fear for another and you call it hope. I signed forms I did not fully understand. I would sign them again.

4 So here is what it is to read, in a corridor I have memorized, that a team at Great Ormond Street and University College London has grown an esophagus in a laboratory, two and a half centimeters of it, from a pig stripped of its own cells and reseeded with the recipient's, so that the body would not need to be drugged into accepting it, and that in eight small pigs the thing took, grew its own muscle and nerves and blood, let them swallow and grow at a normal rate, with no rejection because there was nothing foreign left to reject. Five of the eight were still well at six months. The work was funded by a hospital charity and published in March. [[4]]I read it twice on my phone and then I put the phone face down on the windowsill.

5 I want to be careful with hope; I have been burned by it. The scientists themselves say human trials could be five years off, and five years is a geological age in this corridor. Some of the lab animals developed the same scar-tissue narrowing my daughter knows by name, and it had to be opened up again, the way hers has. So I am not telling you my child is saved. I am telling you that for one evening the trade went away. For one evening I imagined a version of her who eats because she is hungry, who does not carry a pharmacy inside her own immune system, and I let myself sit in that room.

6 She is asleep now, two tiles down, the feed running quiet. I do not know what her life will be. But somewhere a pig grew a throat that was its own, and a stranger I will never meet signed the grant that paid for it, and that is closer than we were on Friday. I will be in this chair when she wakes.

AP-style multiple choice:

1. In the first paragraph, the writer cites the figure of roughly one in four thousand and then says she stopped finding it comforting because a number does not feed anybody. This sequence primarily serves to
 - A) introduce the scientific breakthrough that the rest of the opener will explain
 - B) prove that esophageal atresia is rarer than readers would assume and is therefore underfunded
 - C) argue that medical statistics are generally unreliable and should be distrusted by patients
 - D) establish that the writer has lived the situation while signaling that statistics fall short of the daily reality she describes
2. Which of the following best characterizes the writer's tone toward the breakthrough across the final three paragraphs?
 - A) detached and analytical, weighing the research as a neutral observer would
 - B) bitter and resentful that the treatment did not arrive sooner for her child
 - C) triumphant and certain that her daughter's condition will soon be cured
 - D) guarded and self-protective, allowing herself hope only briefly and on stated conditions

Discussion questions:

1. The writer says of the immunosuppression decision, You trade one fear for another and you call it hope. How does that line shape the way you read her response to the new research later in the opener, and what does it reveal about what hope costs her?
2. This opener is written entirely from one parent's point of view, with no doctor or scientist given their own voice. What does the first-person limitation gain for the piece, and what might a more reported version include that this one leaves out?

Writing prompt (Homework or extended in-class, Q2 rhetorical analysis):

The following passage is a first-person opener written in the voice of a parent whose child was born with esophageal atresia, responding to news of a laboratory-grown esophagus that may one day be transplanted without anti-rejection drugs. Read the passage carefully. Then write an essay that analyzes the rhetorical choices the writer makes to convey her complicated response to the breakthrough. In your response you should develop your analysis with specific, well-chosen evidence from the passage; consider how choices such as the writer's concrete imagery, sentence structure, handling of statistics, and concessions shape the reader's understanding of her guarded hope; and explain how these choices serve her purpose. Avoid mere summary or simple device-spotting; tie each choice to its effect on the reader.